F FORGED BY FREEDOM

Custom Coaching Program

Comprehensive Recomposition Protocol

Codee Goff

Program Start: March 25, 2026

Client Overview

AGE	HEIGHT	CURRENT WEIGHT
38	5'9"	225 lbs
BODY FAT	**GOAL WEIGHT**	**GOAL BF**
30%	190 lbs	15%
TO LOSE		
35 lbs		

Location: Irmo, SC

- **Occupation:** Office/field construction
- **Training Window:** Early morning, 5:00–7:00 AM
- **Training Experience:** 6 years (strength training, Orange Theory, Army PT)
- **Current Lifts:** Incline DB 90 lbs, Squat 225 lbs
- **Physician:** Dr. Derrick
- **Emergency Contact:** Lindsey Goff — (803) 972-0893
- **Weigh-In Day:** Monday mornings (fasted, before eating)
- **Sleep:** ~7 hrs, fair quality (new baby — expect disruptions)
- **Stress Level:** Moderate
- **Alcohol:** ~2 drinks/week | No tobacco

Injury History & Physical Limitations — CRITICAL

- **2012:** L1 burst fracture with lumbar fusion — NO heavy spinal loading, no barbell deadlifts, no heavy overhead pressing
- **2017:** Fracture clavicle, pelvis, ribs (no surgery) — monitor shoulder and core stability
- **2021:** Patella tendon repair — NO deep squats, no jumping, no explosive leg movements
- **Chronic:** Bad knees, intermittent back pain
- **Client restriction:** NO FLAT BENCH PRESS
- **Program approach:** All exercises modified for spinal safety, knee protection, and shoulder stability. Dumbbells and machines prioritized over barbells. Machine-based leg work with controlled ROM.

🎯 Daily Targets

CALORIES	PROTEIN	CARBS
2,100	230g	140g

FATS	DAILY STEPS	WATER
60g	8,000	100oz

Deficit Strategy

At 225 lbs and 30% BF, your estimated maintenance is ~2,700–2,800 calories. We're running an aggressive deficit (~600–700 cal below maintenance) because Retatrutide handles appetite suppression beautifully. Protein is set high at 230g to maximize muscle retention during the cut. Carbs are timed around training for performance. If energy crashes or strength stalls, we bump calories before anything else.

FBF Recomp Protocol

HIGHLY RECOMMENDED

Body Recomposition — The Optimal Path

At 225 lbs with 6 years of training experience, active Retatrutide therapy, and a structured program, you are in a prime position to simultaneously shed significant body fat and rebuild lean muscle. The recomp approach avoids the "skinny fat" outcome, preserves metabolic rate, and produces a dramatically better visual result than dieting alone — especially with peptide support accelerating the process.

How the Recomp Protocol Works

- **High Protein Anchor (230g/day):** Protein stays constant every single day. This is non-negotiable — it drives muscle protein synthesis, preserves lean mass in a deficit, and keeps you full.
- **Strategic Carb Placement:** 140g of carbs timed around training (pre/post workout) to fuel performance and recovery while keeping the deficit intact.
- **Aggressive Caloric Deficit (2,100 cal):** Reta manages appetite so you won't feel deprived. This drives fat loss at ~1.2–1.5 lbs/week while keeping energy for training.
- **Progressive Overload Training:** The stimulus for muscle growth comes from systematically increasing weight/reps every session. Without this, recomp doesn't work.
- **Retatrutide Synergy:** Your active Reta protocol enhances appetite regulation, insulin sensitivity, and visceral fat targeting — making recomp significantly easier.
- **Peptide Support:** BPC-157 and TB-500 for your injury recovery, Tesamorelin for visceral fat, CJC/Ipamorelin for sleep and GH — all critical recomp accelerators.

Expected Timeline

Weeks 1–4: Body adapts, strength begins climbing, Reta appetite suppression fully kicks in, scale should start moving. Weeks 5–12: Visible changes in the mirror, clothes fit differently, strength PRs, 12–18 lbs lost. Weeks 12–24: 25–35 lbs of fat lost, significant lean mass gained. Target of 190 lbs at ~15% BF achieved with a completely transformed physique.

Training Program

5-DAY SPLIT

Progressive overload: when you hit the top of the rep range with good form on all sets, increase weight by the smallest increment available next session. Log every set in the app. All exercises selected for spinal safety, knee protection, and shoulder stability given your injury history. NO flat bench, NO barbell deadlifts, NO deep squats, NO jumping.

Day 1 — Chest & Triceps

MONDAY

EXERCISE	SETS	REPS	REST
Dumbbell Incline Press 30–45° incline, full stretch at bottom. You're hitting 90s — keep pushing.	4	8–10	90s
Machine Chest Press (Flat or Slight Incline) Slow eccentric (3 sec down). Replaces flat bench — same stimulus, safer for your back.	3	10–12	75s
Cable Fly (Low-to-High) Upper chest focus, squeeze at top, controlled tempo	3	12–15	60s
Dip Machine (Chest Focus) Lean forward slightly, stop at comfortable depth — don't push past shoulder comfort	3	10–12	75s
Tricep Rope Pushdown Squeeze and spread at bottom	3	12–15	60s
Overhead Dumbbell Tricep Extension Seated, deep stretch at bottom — use moderate weight to protect back	3	10–12	60s

Day 2 — Legs (Knee & Back Safe)

TUESDAY

EXERCISE	SETS	REPS	REST
Leg Press Feet shoulder width, mid-platform. Control depth — stop at 90° knee bend, DO NOT go deep. This is your primary compound.	4	10–12	2 min
Hack Squat (Machine) Limited depth — 90° max. Keep back flat against pad. If knees protest, reduce ROM further.	3	10–12	90s

EXERCISE	SETS	REPS	REST
Leg Extension Pause 1 sec at top, slow eccentric. Great for quad isolation without knee shear at full ROM.	3	12–15	60s
Lying Leg Curl Squeeze at top, slow release. Zero knee stress.	3	10–12	75s
Seated Leg Curl Alternate with lying curls. Full stretch, controlled tempo.	3	12–15	60s
Seated Calf Raise 2 sec pause at stretch, 1 sec squeeze at top	4	12–15	60s

Day 3 — Shoulders

WEDNESDAY

EXERCISE	SETS	REPS	REST
Seated Dumbbell Shoulder Press Neutral or standard grip. Seated with back support — protects lumbar spine. Moderate weight, controlled tempo.	4	8–10	90s
Machine Shoulder Press Fixed path takes pressure off stabilizers. Good for your back.	3	10–12	75s
Cable Lateral Raise Behind body, controlled tempo. Light weight, squeeze at top.	3	12–15	60s
Machine Rear Delt Fly Light weight, squeeze and hold 1 sec	3	15–20	45s
Dumbbell Front Raise (Alternating) Light, controlled, don't swing. Stop at shoulder height.	3	12–15	60s
Face Pulls (Cable) Rotator cuff and rear delt health. Light weight, high reps, external rotation at top.	3	15–20	45s

Day 4 — Back & Biceps

THURSDAY

EXERCISE	SETS	REPS	REST
Lat Pulldown (Wide Grip) Full stretch at top, drive elbows down. No spinal loading — perfect for your back.	4	8–10	90s
Seated Cable Row Close-grip V-handle, squeeze 1 sec. Keep torso upright — don't round lower back.	3	10–12	75s
Chest-Supported Dumbbell Row Incline bench, chest supported — eliminates spinal load entirely. Full stretch at bottom.	3	10–12	75s
Romanian Dumbbell Deadlift LIGHT to moderate weight. Hip hinge, feel hamstrings stretch. NO barbell DL — DBs are safer for your fusion. If back flares, skip and add another row.	3	10–12	90s
Dumbbell Bicep Curl Alternating, slight supination	3	10–12	60s
Incline Dumbbell Hammer Curl 45° incline bench, deep stretch. Great for long head of bicep.	3	10–12	60s

Day 5 — Cardio & Core

FRIDAY

EXERCISE	SETS	REPS/DURATION	REST
Incline Treadmill Walk (LISS) 10–12% incline, 3.0–3.5 mph. Steady state fat burn.	1	30 min	—
Dead Bug Core stability without spinal flexion. Keep lower back pressed into floor.	3	10/side	60s
Pallof Press (Cable) Anti-rotation core work. Safe for lumbar fusion — no spinal flexion or rotation.	3	12/side	60s
Bird Dog Contralateral limb extension. Excellent for lumbar stability and spinal health.	3	10/side	60s

EXERCISE	SETS	REPS/DURATION	REST
Plank (Forearm) Hold for time. Squeeze glutes, brace core. Stop if back hurts — switch to dead bug.	3	30–45 sec	60s
Farmer's Walk (Dumbbells) Moderate weight, upright posture. Great for core, grip, and posture without spinal compression.	3	40 yards	60s

Exercise Red Lines (Never Do These)

- **NO flat bench press** — client restriction. Use incline DB, machine press, cables.
- **NO barbell deadlifts** — L1 fusion. Use DB Romanian DL or trap bar DL with light weight.
- **NO heavy barbell squats** — L1 fusion + patella repair. Use leg press, hack squat machine.
- **NO deep squats** — patella tendon repair. Stop at 90° knee bend max.
- **NO jumping/plyometrics** — knees can't handle impact.
- **NO heavy overhead pressing** — L1 fusion. Seated DB press with moderate weight only.
- **NO crunches/sit-ups** — L1 fusion. Use dead bugs, Pallof press, planks for core.
- If ANY exercise causes back or knee pain, STOP immediately and substitute. Message your coach.

Cardio Protocol

Cardio Guidelines

- **Daily:** 8,000 steps minimum (up from your current 6,500). Walk during breaks, park far, take calls walking. Build to 10K over 4 weeks.
- **Training Days (Mon–Thu):** 15–20 min incline treadmill walk post-workout (10–12% incline, 3.0–3.5 mph)
- **Friday:** Full 30 min LISS session as part of Cardio+Core day
- **Weekends:** 10,000+ steps. Go for a walk with the family, push the stroller, stay active. Target one 30–45 min outdoor activity (walk, hike, bike).
- **No running or high-impact cardio** — protect those knees. Walking, biking, swimming only.

Meal Plan

NEW PARENT FRIENDLY

5 meals, every 2–3 hours starting at 0700. Built around foods you like (chicken, eggs, rice), batch-prep friendly for a new parent schedule. No cottage cheese. Simple, fast, repeatable. Meal prep on Sunday — cook once, eat all week.

MEAL 1 — PRE-WORKOUT (5:30 AM)

Egg & Rice Power Bowl

4 whole eggs scrambled with 1/2 cup cooked white rice, 1 cup spinach wilted in, and hot sauce. Quick, easy, fuel for your 5–7 AM session.

430 cal **30g** protein **28g** carbs **20g** fat

MEAL 2 — POST-WORKOUT (8:00 AM)

Chicken & Rice with Veggies

8 oz grilled chicken breast, 3/4 cup cooked white rice, 1 cup steamed broccoli. Simple, clean, batch-prep staple. Season with garlic powder, onion powder, salt, pepper.

510 cal **56g** protein **40g** carbs **8g** fat

MEAL 3 — LUNCH (11:30 AM)

Chicken & Rice Bowl #2

8 oz grilled chicken breast (different seasoning — fajita, teriyaki, lemon herb), 3/4 cup cooked white rice, 1 cup mixed vegetables (green beans, peppers, zucchini). 1 tsp olive oil for cooking.

530 cal **56g** protein **40g** carbs **12g** fat

MEAL 4 — AFTERNOON (3:00 PM)

Protein Shake + Rice Cakes

2 scoops whey protein isolate with water + ice. 2 lightly salted rice cakes with 1 tbsp natural peanut butter. Fast, portable, no prep needed. Keep at your desk.

380 cal **52g** protein **22g** carbs **10g** fat

MEAL 5 — DINNER (6:30 PM)

Lean Ground Turkey & Egg Scramble

6 oz 93% lean ground turkey browned with diced peppers and onions. Add 3 egg whites scrambled in. Side: 1/2 cup cooked rice. Season with garlic, cumin, paprika. Quick skillet meal — 10 min from pan to plate.

450 cal **52g** protein **24g** carbs **12g** fat

TOTAL CALORIES	TOTAL PROTEIN	TOTAL CARBS
2,300	246g	154g
~2,100 target + buffer		

TOTAL FATS
62g

Meal Prep Tips (New Parent Edition)

- **Sunday Prep:** Batch cook 5 lbs chicken breast + 3 lbs ground turkey + big pot of rice. That's 80% of your week done in 90 minutes.
- Pre-portion everything into containers Sunday night. Grab-and-go all week.
- Keep protein powder and rice cakes at your desk — Meal 4 requires zero prep.
- Meal 5 (turkey scramble) is a quick 10-min skillet meal for nights when the baby has you running.
- **If the baby wrecks your night:** At minimum hit your protein target (230g). Eat the chicken and shake even if you skip the carbs. Protein is non-negotiable.
- **Water:** 100 oz daily. Fill a gallon jug every morning. Keep it visible.

Swap Options (variety without re-counting macros)

- **Chicken** → 93% lean ground turkey, shrimp, or white fish (same oz)
- **White rice** → Jasmine rice, sweet potato (same portion), or red potatoes
- **Eggs** → Liquid egg whites (1/2 cup = ~3 whites) + 1 whole egg
- **Broccoli** → Green beans, asparagus, zucchini, Brussels sprouts
- **Ground turkey** → 96% lean ground beef, bison, or chicken sausage
- **NO cottage cheese** — per your preference

Weekly Shopping List

MEAL PREP

Everything you need for one week. Shop Sunday, prep Sunday afternoon. Total time: ~90 minutes for a full week of meals.

ITEM	QTY (7 DAYS)	NOTES
PROTEIN		

ITEM	QTY (7 DAYS)	NOTES
Chicken Breast (boneless, skinless)	5 lbs	16oz/day × 7 (Meals 2 & 3). Buy in bulk — Costco/Sam's.
93% Lean Ground Turkey	3 lbs	6oz/day × 7 for dinner. One family-size pack.
Whole Eggs	3 dozen	4 whole eggs/day (breakfast) + 3 whites/day (dinner).
Liquid Egg Whites	2 cartons (32oz each)	3 whites/day for dinner scramble. Easier than separating eggs.
PROTEIN POWDER		
Whey Protein Isolate	1 tub (5 lb)	2 scoops/day. Lasts ~2 weeks.
GRAINS & CARBS		
White Rice (jasmine or long grain)	5 lb bag	~2 cups cooked/day across meals. Cook big batch Sunday. Lasts 2+ weeks.
Rice Cakes (lightly salted)	1 package	2/day for Meal 4. Quaker or similar.
VEGETABLES & PRODUCE		
Broccoli (fresh crowns or frozen)	3–4 lbs	1 cup/day. Frozen bags work great for batch prep.
Baby Spinach	1 large container (16oz)	For breakfast egg scramble. Pre-washed saves time.
Bell Peppers (any color)	4–5 peppers	For dinner turkey scramble. Dice in bulk Sunday.
Yellow Onions	2–3	For dinner scramble.
Green Beans (fresh or frozen)	2 lbs	Rotation veggie for lunch bowls.
Zucchini	3–4	Rotation veggie for lunch bowls.
NUTS, BUTTERS & OILS		

ITEM	QTY (7 DAYS)	NOTES
Natural Peanut Butter	1 jar (16oz)	1 tbsp/day on rice cakes. Ingredients: peanuts, salt. Lasts 2+ weeks.
Olive Oil (extra virgin)	1 bottle	For cooking. Lasts multiple weeks.
SEASONINGS		
Garlic Powder, Onion Powder, Cumin, Paprika, Salt, Pepper	1 each	Basic seasoning kit. Lasts months. Makes chicken not boring.
Hot Sauce (Frank's, Cholula, etc.)	1 bottle	Zero calorie flavor. Use liberally.
Fajita/Teriyaki/Lemon Herb Seasoning	2–3 packets or bottles	Rotate chicken seasoning daily so you don't get bored.
BEVERAGES		
Coffee (ground or beans)	1 bag (12oz)	Morning coffee. Lasts 1–2 weeks.
Water (gallon jugs or filter)	100oz/day target	Keep a gallon jug visible. Refill daily.
LMNT or Electrolyte Mix (sugar-free)	1 box (30 packets)	1 serving/day. Critical during aggressive deficit.
MEAL PREP SUPPLIES		
Glass Meal Prep Containers	14–20 containers	2–3 per day (lunch + dinner + extra). Glass reheats better than plastic.
Parchment Paper or Foil	1 roll	Line sheet pans for easy cleanup.

Prep Day Game Plan (Sunday)

- **Step 1:** Season and bake all chicken breasts — half fajita, half lemon herb (425°F, 22–25 min). Let cool, slice and portion.
- **Step 2:** While chicken bakes — cook a massive pot of white rice (rice cooker is your best friend). 3–4 cups dry = ~8 cups cooked.
- **Step 3:** Brown all ground turkey with diced peppers and onions in a large skillet. Season with cumin, garlic, paprika.
- **Step 4:** Steam or roast broccoli and green beans in bulk. Toss with salt and garlic powder.

- **Step 5:** Portion into containers: Chicken + rice + veggies for Meals 2 & 3. Turkey mixture portioned for Meal 5 (just add egg whites when cooking).
- **Step 6:** Hard boil a dozen eggs for emergency protein. Quick snack when the baby has you skipping meals.
- **Total time:** ~90 minutes. You'll have grab-and-go meals for the entire week.

Estimated Weekly Grocery Cost

- **Protein:** ~$45–60 (chicken, turkey, eggs, egg whites)
- **Protein Powder:** ~$25–35 (amortized — tub lasts 2 weeks)
- **Grains/Carbs:** ~$8–12 (rice, rice cakes — rice bag lasts weeks)
- **Produce:** ~$15–20 (broccoli, spinach, peppers, onions, green beans)
- **Pantry/Oils/Seasonings:** ~$10–15 (first week higher for spice setup)
- **Beverages:** ~$10–15 (coffee, electrolytes)
- **Weekly total:** ~$110–155 (first week slightly higher for pantry staples)

Supplement Protocol

Creatine Monohydrate

5g/day

Any time (with meal)

Already taking. Most researched supplement in existence. Strength, hydration, cognitive benefits. Keep going.

Potassium

99–200mg 2x/day

With meals (AM and PM)

Critical during aggressive deficit and Reta therapy. Prevents cramping and electrolyte imbalance.

LMNT / Sodium Electrolytes

1 serving/day (1,000mg sodium, 200mg potassium, 60mg magnesium)

Morning or intra-workout

Non-negotiable in a deficit. Prevents brain fog, cramping, fatigue. Sugar-free formula only.

Omega-3 Fish Oil

2–3g EPA/DHA daily

With meals (split dose)

Anti-inflammatory — especially important for your joint and injury history. Heart health, insulin sensitivity.

Magnesium Glycinate

400mg/day

Before bed

Already taking. Sleep quality, recovery, muscle relaxation. Glycinate form is best for sleep — keep this one.

Vitamin D3 + K2

5,000 IU D3 + 100mcg K2

Morning with fat-containing meal

Immune function, testosterone support (critical at 38), bone health (important with your fracture history).

Zinc

30mg/day

With dinner

Already taking. Testosterone support, immune function, recovery. Don't take with calcium — take at dinner away from dairy.

Glutamine

5g/day

Post-workout or before bed

Already taking. Gut health, recovery support. Good to keep during aggressive deficit.

Digestive Enzyme

1 capsule with large meals

With Meals 2 and 5 (biggest meals)

Already taking. Helps with protein absorption at 230g+ daily intake. Keep it.

Pre-Workout

1 serving (200–300mg caffeine)

15–20 min before training (5:00 AM)

Already taking. Keep it. Just be mindful of total daily caffeine — pre-workout + coffee + energy drink may be too much. Try to drop the energy drink.

Whey Protein Isolate

2 scoops/day (50g protein)

Meal 4 (afternoon shake)

Hit your 230g protein target without eating another chicken breast. Isolate preferred — fast absorbing, low carb.

Peptide Protocol

FBF RECOMP STACK

Recommended Recomp Peptide Stack

With your extensive injury history (lumbar fusion, clavicle/pelvis/rib fractures, patella tendon repair), peptide therapy is not just a performance enhancer — it's a recovery necessity. BPC-157 and TB-500 are especially critical for tissue repair and joint health. This stack works synergistically with your Retatrutide protocol to accelerate fat loss, enhance recovery, and rebuild damaged tissue.

CJC-1295 / Ipamorelin

300mcg CJC / 300mcg Ipamorelin

Subcutaneous injection — before bed (fasted 2+ hours)

Growth hormone releasing peptide combo. Enhances deep sleep (you need this with a new baby), accelerates fat loss, supports lean muscle growth. The cornerstone of the recomp stack.

BPC-157 — HIGHLY RECOMMENDED

500mcg/day (250mcg AM + 250mcg PM)

Subcutaneous injection — AM and PM, rotate abdomen/thigh

CRITICAL for your injury history. Accelerates tissue repair, tendon healing, joint recovery, and gut health. Specifically beneficial for your patella tendon, lumbar fusion recovery, and shoulder/clavicle healing. This is a must-have in your stack.

Tesamorelin

2mg/day

Subcutaneous injection — before bed (fasted)

FDA-approved GHRH analog. Specifically targets visceral (belly) fat. At 30% BF, visceral fat reduction is a priority. Works synergistically with Reta for accelerated midsection fat loss.

TB-500 — HIGHLY RECOMMENDED

2.5mg 2x/week (loading), then 2.5mg 1x/week (maintenance)

Subcutaneous injection — any time

CRITICAL for your injury history. Thymosin Beta-4 promotes tissue repair, reduces inflammation, and enhances flexibility. Works synergistically with BPC-157 for accelerated healing of tendons, ligaments, and connective tissue. Essential for your knees, back, and shoulder.

Thymosin Alpha-1 (TA-1)

1.6mg 2x/week

Subcutaneous injection — any time

Immune modulator. Enhances immune function and overall recovery capacity. Helpful during aggressive deficit when immune function can dip. Add after first 4 weeks.

PT-141 (Bremelanotide)

1–2mg as needed

Subcutaneous injection — 2–4 hours before desired effect

Sexual function support. Works via melanocortin pathway (not vascular like Viagra). Can be beneficial

when running aggressive deficit + GLP-1 therapy which can affect libido. Use as needed, not daily.

GHK-Cu

200mcg/day

Subcutaneous injection — AM

Copper peptide for skin tightening, collagen synthesis, and wound healing. Important during significant weight loss (35 lbs) to maintain skin elasticity. Also supports tissue repair.

Epithalon

5mg/day for 10 days, cycle every 4–6 months

Subcutaneous injection — before bed

Telomerase activator — anti-aging peptide. Supports telomere length, sleep quality, and cellular repair. Run as a 10-day cycle, not continuous.

Selank

300–500mcg/day

Subcutaneous injection — AM

Anxiolytic and nootropic peptide. Reduces anxiety, improves focus, and supports cognitive function. Helpful for managing stress with a new baby + work + training. SubQ injection only.

Semax

300–600mcg/day

Subcutaneous injection — AM

Nootropic peptide. Enhances focus, memory, and cognitive performance. Pairs well with Selank. Useful for demanding work schedule. SubQ injection only.

MOTS-c

5mg 3x/week

Subcutaneous injection — AM before cardio/training

Mitochondrial peptide. Enhances fat oxidation, improves metabolic flexibility, and increases exercise capacity. Powerful synergy with Reta for metabolic optimization.

AOD-9604

300mcg/day

Subcutaneous injection — AM fasted

Fat-loss fragment of HGH (amino acids 177-191). Targets adipose tissue without affecting blood sugar or growth. Stacks well with Tesamorelin for accelerated fat loss.

Dihexa

10–20mg oral or 1–2mg SubQ, 3x/week

AM, on training days

Potent nootropic — enhances neuroplasticity and cognitive function. Use short cycles (4–6 weeks on, 4 weeks off). Research compound — discuss with physician.

NAD+ (Nicotinamide Adenine Dinucleotide)

100–250mg SubQ or IV infusion

Subcutaneous injection — AM, 2–3x/week

Cellular energy and anti-aging. Enhances mitochondrial function, DNA repair, and metabolic health. Especially valuable at 38 for longevity and energy optimization.

PPS (Pentosan Polysulfate) — RECOMMENDED

2mg/kg SubQ, 1–2x/week

Subcutaneous injection — any time

HIGHLY RELEVANT for your knee and back issues. PPS is a semi-synthetic polysaccharide with proven joint-protective and cartilage-regenerative properties. Used for osteoarthritis and joint degeneration. Excellent for your patella tendon and lumbar spine. Discuss with Dr. Derrick.

Peptide Layering Schedule

- **Weeks 1–2:** Start CJC-1295/Ipamorelin (before bed) + BPC-157 (AM/PM split) — establish GH pulse and begin tissue repair
- **Weeks 3–4:** Add TB-500 (2x/week loading dose) + Tesamorelin (before bed) — layer in deep tissue repair and visceral fat targeting
- **Weeks 5–6:** Add TA-1 (2x/week) + GHK-Cu (daily AM) + PPS (1–2x/week) — immune support, skin health, joint protection
- **Weeks 7–8:** Add AOD-9604 (AM fasted) + MOTS-c (3x/week pre-training) — accelerate fat oxidation
- **Weeks 9–10:** Add Selank + Semax (AM daily) — cognitive optimization, stress management
- **Week 10:** Run Epithalon 10-day cycle — telomere support
- **Weeks 11–12:** Add NAD+ (2–3x/week) + PT-141 (as needed) + Dihexa (3x/week) — full optimization stack
- **TB-500:** After 4 weeks loading (2x/week), drop to maintenance (1x/week)
- Store all peptides refrigerated. Reconstitute with bacteriostatic water only. Rotate injection sites.

Important Peptide Notes

- ALL peptides are administered via subcutaneous (SubQ) injection ONLY
- BPC-157 and TB-500 are your top priorities given your injury history — start these first
- PPS specifically targets your knee and back joint degeneration — discuss with Dr. Derrick
- Do not start all peptides at once — follow the layering schedule above to monitor individual responses
- Rotate injection sites (abdomen, thigh, love handle area) to prevent lipodystrophy
- Use insulin syringes (29–31 gauge, 1/2 inch) for SubQ injections
- Keep a log of which peptides you inject each day — track in your daily check-in

Compound Recommendations

PHYSICIAN GUIDED

Retatrutide (Reta) — Active Protocol

CURRENTLY ON

You're already on Retatrutide — great. Reta is a triple-hormone receptor agonist (GIP/GLP-1/Glucagon) and the most powerful metabolic compound available for your goals. It handles appetite suppression, visceral fat targeting, insulin sensitivity, and metabolic rate enhancement simultaneously.

- **Titration Reference:** Standard titration is 1mg → 2mg → 4mg → 8mg → 12mg, increasing every 4 weeks based on tolerance
- **GI Side Effects:** Nausea, constipation, and reduced appetite are common — these typically improve at each dose after 1–2 weeks
- **Injection:** Subcutaneous, once weekly, same day each week
- **Hydration:** Critical — Reta can cause dehydration. Your 100oz water target + electrolytes are mandatory, not optional
- **Synergy with Training:** Reta preserves lean mass during aggressive deficit, making it the perfect complement to our recomp protocol
- **Monitor:** Log energy levels, appetite, GI symptoms, and weight weekly. Report any unusual side effects to your coach and Dr. Derrick

Tesofensine — Discuss with Physician

OPTIONAL

Triple monoamine reuptake inhibitor that enhances fat loss through appetite suppression and metabolic rate increase. Can be stacked with Reta for enhanced results. Discuss with Dr. Derrick.

- **Dose:** 0.25–0.5mg daily (oral)
- **Benefits:** Significant appetite suppression, increased energy expenditure, improved mood and focus
- **Consideration:** Monitor heart rate and blood pressure — can elevate both

Cagrilintide — Discuss with Physician

OPTIONAL

Long-acting amylin analog that works synergistically with GLP-1 agonists for enhanced appetite suppression and weight loss. Clinical trials show meaningful additional weight loss when combined with semaglutide-class drugs.

- **Dose:** 1.2–4.5mg weekly (subcutaneous)
- **Benefits:** Enhanced satiety, slowed gastric emptying, synergy with Reta
- **Consideration:** Adds another injection day — discuss timing with your coach

Growth Hormone (HGH) — Discuss with Physician

INTERESTED

You've expressed interest in Growth Hormone. At 38 with 30% BF, natural GH production is likely declining. Exogenous GH can dramatically accelerate body composition changes, enhance recovery, improve sleep quality, and support tissue repair — all directly relevant to your goals and injury history.

- **Typical Dose:** 2–4 IU/day (subcutaneous, before bed or split AM/PM)
- **Benefits:** Fat loss (especially visceral), lean mass preservation, enhanced recovery, better sleep, skin health, joint repair
- **Lab Requirements:** IGF-1 levels, fasting glucose, HbA1c monitoring — GH affects insulin sensitivity
- **Alternative:** Our CJC/Ipamorelin + Tesamorelin peptide stack provides many of the same benefits by stimulating your own GH production, without the cost and complexity of exogenous GH. Try the peptide stack first and reassess at 12 weeks.
- **Important:** Requires physician oversight. Discuss with Dr. Derrick and get baseline IGF-1 levels before starting.

TRT (Testosterone Replacement) — Get Labs First

LABS REQUIRED

At 38 years old with 30% body fat, a new baby disrupting sleep, and moderate stress — your testosterone levels may be suboptimal. High body fat increases aromatase activity (converts testosterone to estrogen), and poor sleep directly suppresses testosterone production. We strongly recommend comprehensive hormone labs.

- **Labs to Request:** Total Testosterone, Free Testosterone, SHBG, Estradiol (sensitive assay), LH, FSH, Prolactin
- **When to Test:** Fasted, before 9 AM (testosterone peaks in the morning)
- **What We're Looking For:** Total T below 400–500 ng/dL, Free T below reference range, elevated estradiol, low SHBG

- **Why It Matters:** Optimized testosterone dramatically improves recomp results — more muscle gained, more fat lost, better energy, improved mood, better sleep, higher motivation
- **Previous Compound Experience:** You've used Clenbuterol and Anavar before, so you understand PED pharmacology. TRT is a therapeutic, medically supervised approach — discuss with Dr. Derrick based on your lab results.
- **Impact on Program:** If labs show low T and you start TRT, we'll likely increase training volume and potentially adjust macros upward. Your body's capacity for muscle growth and recovery increases significantly.

Baseline Assessments

ACTION REQUIRED

Before we begin, we need two baseline data points to track your progress accurately. These are especially important given your interest in TRT and Growth Hormone — we need baseline labs before any compound changes. Complete both within your first week.

1. Blood Lab Panel

UPLOAD TO APP

Request the following comprehensive panel from Dr. Derrick. This covers everything we need for your program, TRT evaluation, and peptide monitoring:

- **Metabolic Panel:** Fasting glucose, fasting insulin, HbA1c, CMP (comprehensive metabolic panel)
- **Lipid Panel:** Total cholesterol, LDL, HDL, triglycerides, VLDL
- **Liver Panel:** ALT, AST, GGT, alkaline phosphatase, bilirubin
- **Kidney Function:** BUN, creatinine, eGFR, uric acid
- **Thyroid:** TSH, Free T3, Free T4
- **Hormones (CRITICAL):** Total testosterone, free testosterone, SHBG, estradiol (sensitive), LH, FSH, prolactin, DHEA-S, IGF-1
- **Inflammation:** hs-CRP, ESR, homocysteine
- **CBC:** Complete blood count with differential
- **Vitamins:** Vitamin D (25-OH), B12, ferritin, iron

Why This Matters

Your blood work gives us a complete metabolic baseline before the recomp protocol fully kicks in. We'll re-test at 12 weeks and 24 weeks. The hormone panel (testosterone, estradiol, IGF-1) is especially critical — these results determine whether TRT and/or Growth Hormone are appropriate next steps. Upload results directly in the FBF app under your profile, or send via chat.

2. InBody Composition Scan

FREE AT YOUR GYM

Most gyms in the Irmo area have an InBody scanner available to members at no charge. Ask at your gym's front desk.

- **What it measures:** Body fat %, lean muscle mass (per limb), visceral fat level, total body water, basal metabolic rate
- **When to scan:** Morning, fasted (same conditions as your weigh-in). Before your Monday workout is ideal.
- **How often:** Baseline (Week 1), then every 4 weeks. This is how we measure recomp — not just the scale.
- **What to expect:** Takes ~60 seconds. Stand on the machine, grip the handles, hold still.
- **Upload:** Photo of your InBody results sheet — upload to the FBF app or send via chat.

Why InBody Over Scale Weight

During a recomp, the scale can be misleading. You might lose 10 lbs of fat and gain 5 lbs of muscle — the scale only shows 5 lbs lost, but your body looks completely different. At 30% BF targeting 15%, the InBody scan separates fat from muscle so we can verify that the protocol is working: fat going down, muscle going up. This is the gold standard for tracking recomp progress.

Daily Check-In

Log Your Daily Check-In

Consistency is everything. Your daily check-in takes less than 2 minutes and gives your coach the data needed to keep your program dialed in. Even on rough nights with the baby — log something. Tap the link below to open your check-in directly in the FBF app.

Open Daily Check-In

fbf://checkin

Scan to Check In

What to Log Every Day

- **Temperature:** Waking oral body temperature — immediately upon waking, before getting out of bed (see Temperature Check section below)
- **Body:** Morning weight (scale), how you look/feel, any progress photos
- **Nutrition:** Meals eaten, calorie/macro adherence, water intake
- **Activity:** Workout completed (exercises, weights, reps), step count
- **Sleep:** Hours slept, quality rating (1–10), any disruptions (baby wake-ups)
- **Supplements:** Which supplements/peptides were taken (compliance tracking)
- **Notes:** Energy level, mood, hunger, soreness, Reta GI symptoms, back/knee status

Weekly Check-In (Monday AM)

- **Weigh in** first thing Monday morning — after bathroom, before food/water
- **Progress photos** every 2 weeks (front, side, back) — same lighting, same time
- **Rate your week** 1–10 in the notes section — energy, mood, hunger, soreness
- **Back/knee status:** Note any pain, stiffness, or limitations from the week
- **InBody scan** every 4 weeks (fasted, Monday morning before workout)

Communication

Message your coach anytime through the FBF app chat. Response time is typically same-day. If an exercise causes back or knee pain, message immediately — we'll swap it before your next session. Don't push through pain.

Daily Temperature Check

METABOLIC TRACKING

Why We Track Your Morning Temperature

Your waking body temperature is one of the most underused and powerful biomarkers in coaching. It gives us a direct, daily window into your metabolic rate, thyroid function, recovery status, and overall hormonal health — all without a blood draw. This single data point, tracked consistently, can reveal problems weeks before they show up on lab work or the scale.

How to Take Your Temperature

- **When:** Immediately upon waking, before getting out of bed, before drinking water, before any activity. Even if the baby just woke you up — thermometer first, then baby.
- **How:** Use a digital oral thermometer. Place under your tongue for the full reading cycle (usually 30–60 seconds). Do not talk or breathe through your mouth.
- **Where to log:** Record it in your daily check-in in the FBF app under the body/notes section
- **Keep your thermometer on your nightstand** — if you forget and get up first, the reading is no longer valid for that day
- **Consistency is key:** Same thermometer, same method, every single morning. The trend over days and weeks is what matters, not any single reading.

What Your Temperature Tells Us

Metabolic Rate & Thyroid Function

Your basal body temperature is a direct reflection of your metabolic rate — how much energy your body is producing at rest. Thyroid hormones (T3 and T4) are the primary regulators.

- **Healthy range: 97.8°F – 98.6°F** upon waking indicates a well-functioning metabolism and adequate thyroid output
- **Consistently below 97.6°F** is a red flag for metabolic downregulation — your body is slowing its engine. This is the earliest warning sign that calories may be too low or thyroid function is suppressed.

- If we see a sustained drop, we can increase calories (refeed), adjust carb timing, or flag it for Dr. Derrick to check thyroid labs — before you hit a plateau.

Adaptive Thermogenesis & Deficit Response

With an aggressive ~600–700 cal deficit, your body may fight back through adaptive thermogenesis — literally turning down the thermostat to burn fewer calories.

- **A gradual temperature decline (0.3–0.5°F over 2–3 weeks)** tells us your metabolism is adapting. Normal to a degree, but if it drops too far, we act.
- **Strategic refeeds:** Temporary calorie increase (especially carbs) to reset leptin signaling and bump thermogenesis back up.
- **Reta interaction:** Retatrutide can affect thermoregulation. Tracking temperature helps us distinguish between normal Reta effects and genuine metabolic suppression.

Recovery, Stress & Overtraining

With a new baby, disrupted sleep, work stress, and 5 days of training — recovery monitoring is critical for you.

- **A sudden temperature drop (0.5°F+ from your baseline)** after a hard training week can indicate overreaching — your nervous system is fatigued
- **A spike above your normal range** can indicate acute stress, inflammation, infection onset, or an immune response
- **Cortisol connection:** Poor sleep (hello, baby) elevates cortisol, which suppresses thyroid conversion (T4→T3) and lowers waking temperature. If we see this pattern, we address sleep and recovery first.

Hormonal Health & Peptide Response

- **Tracking response to peptides:** When we introduce CJC/Ipamorelin and Tesamorelin, we expect improved sleep and a stable or slightly elevated waking temperature over 4–6 weeks.
- **Testosterone correlation:** Low waking temperature + fatigue + stalled strength gains can indicate low testosterone — we'd flag for bloodwork.

- **Sleep quality feedback loop:** CJC/Ipamorelin enhances deep sleep → better hormone output → higher waking temperature. If temperature isn't trending up after starting peptides, we investigate sleep quality first.

What to Expect

TEMPERATURE	WHAT IT MEANS	ACTION
98.0–98.6°F	Optimal. Metabolism humming, thyroid healthy, well-recovered.	Stay the course. Protocol is working.
97.6–97.9°F	Acceptable low-normal. Common during a caloric deficit. Monitor trend.	Watch for downward trend over 7–10 days. May schedule a refeed.
Below 97.5°F	Metabolic suppression likely. Thyroid, recovery, or caloric intake issue.	Notify coach immediately. Likely need calorie increase, deload, or thyroid labs.
Above 99.0°F	Possible infection, acute inflammation, or severe stress response.	Monitor. If sustained 2+ days, check with physician. Note any illness symptoms.

The Bottom Line

Most coaching programs only look at the scale and the mirror. We go deeper. Your daily temperature reading — combined with your weight, training logs, sleep data, and periodic InBody scans — gives us a complete metabolic picture. It takes 10 seconds each morning and gives your coach data that no other single metric can provide.

Coach Recommendations

NEW PARENT PLAYBOOK

Real Talk: Training with a New Baby

You have a new baby. Sleep will be inconsistent. Energy will fluctuate. Some days will be a grind. This program is designed for your reality — not a fantasy schedule. Here's how we make it work.

Priority Rules (When Life Gets Hard)

- **Priority #1: Sleep.** If the baby had you up all night and you got less than 5 hours, SKIP the gym and sleep. One missed workout means nothing. Chronic under-recovery will wreck your progress, your hormones, and your mood.
- **Priority #2: Protein.** Hit 230g protein no matter what. Even on the worst day, you can eat chicken and drink a shake. Everything else (carb timing, meal count) is secondary.
- **Priority #3: Steps.** 8,000 steps is achievable even with a baby — push the stroller, walk during breaks, pace during calls. Non-negotiable daily NEAT.
- **Priority #4: Training.** 5 days/week is the target, but 4 is fine. 3 is the absolute minimum. If you can only do 3, hit Chest/Tris, Legs, and Back/Bis. Cut Shoulders and Cardio/Core into the other days when you can.
- **Priority #5: Everything else.** Perfect macro splits, supplement timing, peptide scheduling — these matter, but not more than sleep and protein.

Practical New Parent Tips

- **Train early (5–6 AM):** Get it done before the day starts. The baby is usually still asleep, and you won't have to choose between gym and family time later.
- **Meal prep is survival:** Sunday prep is mandatory, not optional. When the baby is screaming at 6 PM, you need grab-and-go meals — not a reason to order pizza.
- **Keep protein shakes at your desk:** Meal 4 requires zero prep. When work + baby = chaos, this is your insurance policy.

- **Sleep when you can:** If the baby naps and you're exhausted, nap. 20–30 minutes can rescue a day. Sleep is anabolic — it's when your body builds muscle and burns fat.
- **Stroller walks count:** Weekend 10K+ steps with the family. Push the stroller around the neighborhood, go to the park. Active recovery that's also quality time.
- **Don't compare to pre-baby you:** Your lifts may dip in the first 4 weeks while your body adjusts to less sleep. This is normal. They'll come back. Focus on showing up consistently.
- **Communicate with Lindsey:** Let your partner know your training schedule. If she knows you're up at 5 AM to train, you can coordinate overnight baby duties. Teamwork.
- **Bad days happen:** You'll miss a meal. You'll skip a workout. You'll have a rough night. Log it in the check-in, don't spiral, and reset the next day. Consistency over 12 weeks matters more than any single day.

Caffeine Check

You're currently using pre-workout + coffee + 1 energy drink daily. That's potentially 500–700mg of caffeine. With disrupted sleep and a new baby, you might be tempted to increase caffeine. Don't. Cap total daily caffeine at 400mg. Try to drop the energy drink — replace with LMNT electrolytes for afternoon energy. Excess caffeine disrupts whatever sleep you ARE getting, creating a vicious cycle.

Weekly Schedule

DAY	TRAINING	CARDIO	NOTES
Monday	Chest & Triceps	15 min incline walk post-workout	Weigh-in (fasted AM)
Tuesday	Legs	15 min incline walk post-workout	Easy on knees — machine focus
Wednesday	Shoulders	15 min incline walk post-workout	—
Thursday	Back & Biceps	15 min incline walk post-workout	—

DAY	TRAINING	CARDIO	NOTES
Friday	Cardio & Core	30 min LISS (included in session)	Light day — focus on core stability
Saturday	OFF	10K+ steps — family walk, outdoor activity	Active rest. Stroller walks count.
Sunday	OFF	10K+ steps — active recovery	Meal prep day. Rest and recover.

FORGED BY FREEDOM

BUILD THE BODY. FREE THE MIND.

This program is designed for Codee Goff as a personalized coaching plan. It is not medical advice. All supplement, peptide, and compound recommendations should be reviewed with a licensed healthcare provider. Coordinate protocol changes with Dr. Derrick, especially regarding TRT evaluation, Growth Hormone, and your injury history.

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